Senate Questions on Notice SQ13-19: Gap Analysis of TGA's Response Against the COVID-19 Vaccine Safety Monitoring Plan

Senate Questions on Notice: SQ13-19 (December 2025)

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Prepared by: Paul Rekaris, Independent Policy Analyst

Executive Summary

The NCIRS 2022 surveillance report documented 160 deaths following COVID-19 vaccination, including six children (median 4.5 days vaccination-to-death). Senator Alex Antic put questions on notice to the Department of Health, Disability and Ageing at the December 2025 Supplementary Budget Estimates (Question on Notice No. 2328, asked 12 December 2025). The question put that only 2 of the 160 deaths were referred to the Vaccine Safety Investigation Group (VSIG), and that the Work Instruction (FOI 4029-06) required the deaths of young people to be referred. The Department answered that none of the nine child deaths on DAEN met the criteria to convene a VSIG, that one fatal case in an 18 to 29-year-old was referred, and that 15 VSIGs reviewed over 50 cases, not all of them deaths. It also described a TGA review of all serious AEFI reports. It produced no records, counts or outputs of that review, and it refused the causality reports on privacy grounds. The response leaves unverified how the remaining deaths were assessed.

Senator Antic asked seven questions about the TGA's COVID-19 vaccine death monitoring (SQ13-19). The response leaves gaps between governance commitments and documented implementation across a four-year investigation period. The Department stated that 15 VSIGs reviewed over 50 cases across the rollout, not all of them deaths. Against 1,053 deaths on DAEN, about 5% or less were therefore reviewed by a VSIG.[1]

The Questions on Notice

The NCIRS 2022 surveillance report documented 160 deaths from 18,398 AEFI reports for COVID-19 vaccines, with median intervals of 4.5 days (children aged 0-17) and 3 days (young adults aged 18-29) between vaccination and death (NCIRS, 2025; Sladden, 2026).

Senator Antic identified:

- Six children (0–17): median vaccination-to-death interval 4.5 days

[1] The Department said the VSIGs reviewed "over 50 cases", not all of which had a death outcome, and did not state how many were deaths. Taking 50 as the figure, 50 of 1,053 is 4.7%. The share of deaths reviewed therefore cannot be stated exactly

- Five young adults (18–29): median vaccination-to-death interval 3 days
- Only 1 of 160 deaths determined as likely linked to vaccination

Seven questions were asked (SQ13–19):

- SQ13: Were the six children referred to VSIG? If not, why not?
- SQ14: Were the five 18–29-year-olds referred to VSIG? If not, why not?
- SQ15: Do causality assessment reports exist for each child death? Provide copies.
- SQ16: Do causality assessment reports exist for each 18–29-year-old death? Provide copies.
- SQ17: Why were only 2 of 160 fatal cases in 2022 referred to VSIG?
- SQ18: How many of 1,053 DAEN deaths have had causality assessments?
- SQ19: How many of 1,053 DAEN deaths been referred to VSIG, and where can reports be accessed?

Key Findings from TGA's Response

- The question refers to 1,053 deaths reported on DAEN. The Department did not state how many received formal causality assessments; it referred questions 18 and 19 back to its answer to questions 13 and 14.
- Since 2021, 15 VSIGs have been convened and have reviewed over 50 cases, not all of them deaths. Against 1,053 deaths, that is about 5% or less.
- DAEN holds 9 reports of death in people under 18. The Department states none met the criteria to convene a VSIG; the assessments were not released, so this cannot be tested.
- Two of the nine child deaths progressed to broader TGA safety investigations (late 2022 and early 2023) that considered population-level evidence, not a VSIG review.
- One fatal report in an 18 to 29-year-old who died in 2022 was referred to a VSIG. The question had identified five 18 to 29-year-old deaths.
- The Department declined to release causality assessment reports, citing patient privacy. It did not say whether they exist.
- The Department listed 10 earlier questions on notice (published July 2023 to April 2024) as previous answers. This audit found none produced Plan implementation documentation.

The analysis identifies a control failure[2] across a four-layer governance hierarchy: National Cabinet Policy passed by Commonwealth Cabinet (comprehensive monitoring commitment) → TGA Safety Monitoring Plan (17 strategies) → VSIG Control Activity (selective expert review) → International Standards (ICH E2E, CIOMS VIII). The activity layer does not demonstrate delivery of the framework requirements, so the governance commitment cannot be shown to be met. For most reported deaths (about 95% or more), no VSIG review is

[2] "Control failure" is the auditor's term. It means either non-fulfilment of an applicable requirement or, where fulfilment cannot be verified from the available documentary evidence, a failure of verification. It makes no finding about intent. "Nonconformity" is used in the sense defined by ISO 19011:2018, cl. 3.21, derived from ISO 9000:2015, cl. 3.6.9 (reference 22). "Governance failure" is used in the same sense.

documented. The Department described a TGA review of all serious AEFI reports but produced no records, counts or outputs of it.

The Governance Failure

National Cabinet required comprehensive COVID-19 vaccine monitoring. The TGA operationalised this through its February 2021 Plan, with 17 documented strategies including Strategy 2.2 (investigation and clinical assessment).

VSIG is a selective control activity within Strategy 2.2. The Work Instruction (FOI 4029-06) convenes it for a serious, unexpected AEFI with no obvious non-vaccine cause that the TGA and OHP agree has “the potential to change the favourable benefit-risk balance”. The Department states that none of the nine child deaths on DAEN met these criteria, and that 15 VSIGs reviewed over 50 cases, not all of them deaths. Measured against the 1,053 deaths in the question, about 5% or less were reviewed by a VSIG. Asked how the rest were assessed, the Department described VSIG as “one of a number of tools” and said all serious AEFI reports are reviewed by the TGA under a standardised process. It produced no records, counts or outputs of that review.

The TGA's AEMDS preliminary fatal case assessment document (FOI 4302-01, p. 5) states that the VSIG should be used only in accordance with its criteria, which it describes as aligned with WHO guidelines, and that it is important to avoid “burning out” the panel. This supports selective referral as a stated approach, but it does not by itself explain the VSIG review rate or establish how the remaining deaths were assessed. This audit has not verified the attribution to WHO guidance. VSIG’s selectivity does not absolve the TGA from reporting against Plan objectives: Strategy 2.2 delivery must be documented regardless of which control mechanism is used. A TGA officer told the Senate on 9 October 2025 that the Plan’s themes “essentially describe our day-to-day processes”, and that there is no single evaluation report benchmarking implementation against the Plan (Hansard, 9 October 2025).

After 10 prior SQONs (July 2023 to April 2024), the TGA has not produced Plan implementation records. The outputs were either not delivered or not documented, and both possibilities are a governance failure in the sense used in this analysis.

Understanding the Governance Framework

The Four Layers

The commitments examined in this analysis operate within a four-layer governance hierarchy where each layer creates binding obligations for the layer below.

Layer 1: National Cabinet Policy (November 2020) established the governance commitment (passed by Commonwealth Cabinet): TGA must "actively and comprehensively monitor" COVID-19 vaccines.

Layer 2: TGA Safety Monitoring Plan (February 2021) operationalised this through 17 numbered strategies, including Strategy 2.2 (investigation and clinical assessment of adverse events of special interest).

Layer 3: VSIG is a selective control activity within Strategy 2.2, designed to provide expert review when specific criteria are met (serious, unexpected, no obvious non-vaccine cause), not to review every death.

Layer 4: International Standards (ICH E2E, CIOMS VIII, ISO 31000) establish the reference framework TGA adopted for documenting systematic pharmacovigilance.

The Critical Distinction

VSIG's selectivity is appropriate by design, but it does not eliminate Strategy 2.2's broader obligation to systematically investigate individual COVID-19 AEFI reports (Strategy 2). When VSIG reviews about 5% or less of deaths, the remaining 95% or more still require documented investigation under Strategy 2.2, yet TGA evidence provided in the Senate indicates that monitoring was not tracked against Plan objectives and cannot produce implementation records after 10 Senate questions on notice.

The Cascading Failure

The control activity (Layer 3) does not demonstrate delivery of the operational framework (Layer 2), so compliance with the governance commitment (Layer 1) cannot be shown, and the practice falls short of the reference standards (Layer 4). The framework was either not delivered as documented or cannot now be independently verified from the available evidence. Both possibilities indicate an evidence-governance failure, while the absence of implementation records prevents verification of compliance with the Commonwealth's implementation of the National Cabinet commitment. The Australian Government's COVID-19 Vaccination Policy had received a first pass by Federal Cabinet before National Cabinet endorsed the policy in November 2020, demonstrating that the policy had been subject to Commonwealth Cabinet consideration. The PM&C *Cabinet Handbook* (para 25) provides that Ministers and portfolio agencies must act on Commonwealth Cabinet decisions. Sladden (2026) reports that the March 2025 policy reversal acknowledged that benefits did not outweigh harms in healthy children, nearly three years after the first child death was reported.

The Obligation to Report

The Australian COVID-19 Vaccination Policy, endorsed by National Cabinet (p.6, p.13), required that TGA's Pharmacovigilance Plan enhance "existing processes for safety signal detection and investigation." This commitment was operationalised by Strategy 2.2 (p.8), which documented a specific obligation to ensure "appropriate and timely investigation of COVID-19 AEFI reports" through dedicated protocols and enhanced investigative capacity. Under the Work Instruction

(FOI 4029-06), a serious AEFI includes an event that results in death, so deaths fall within the scope of this investigative obligation.

TGA designed VSIG as a selective tool, applicable only where events were serious, unexpected, and had no obvious non-vaccine cause. Even accepting that design, TGA retained an obligation to document the investigative pathway applied to non-VSIG cases. The Plan's 17 numbered strategies operationalise Cabinet policy, and TGA must report against them regardless of which tools are used.

Yet VSIG reviewed about 5% or less of reported deaths. For the remaining 95% or more, no VSIG review is documented, and no records of the Department's other assessment process have been produced. Senate evidence indicates that monitoring was not tracked against Plan objectives. This constitutes a governance failure: TGA has not demonstrated Strategy 2.2 delivery for those deaths, and the work is either not done or not documented.

Table 1: Governance Hierarchy Framework

The commitments examined in this analysis operate within a four-layer governance hierarchy. Each layer creates obligations for the layer below. Failure at the control activity level means the higher layers cannot be shown to be met.

Control failure: activity layer (3) does not demonstrate delivery of framework (2), so governance commitment (1) cannot be shown to be met. For about 95% or more of reported deaths, no VSIG review is documented, and the other assessment process described by the Department is not documented.

Layer	Standard / Requirement	TGA Position	Failure Type
1. Governance Commitment National Cabinet Policy (Nov 2020)	"TGA will actively and comprehensively monitor any COVID-19 vaccines for safety" (p.6). "Pharmaco-vigilance Plan" anticipated (p.13, 15). PGPA Act ss 15 and 21 require the accountable authority of a non-corporate Commonwealth entity to govern the entity in a way that is not inconsistent with Australian Government policies.	15 VSIGs reviewed over 50 cases (not all deaths) against 1,053 reported deaths: about 5% or less. For about 95% or more, no VSIG review is documented. Senate Estimates (Oct 2025): evidence indicates monitoring was conducted as "day-to-day processes" and not tracked against Plan objectives.	Strategic misalignment. Control activity does not deliver governance commitment. Consistent with ANAO findings on risk-based targeting and incomplete performance indicators (CPS 107).
2. Control Framework TGA Safety Monitoring Plan (Feb 2021)	17 numbered strategies operationalise policy. Objective 2 (p.5): timely signal detection and investigation. Objective 3 (p.5): timely action on safety concerns (incl. AESI causality). Strategy 2.2 (p.8): individual AEFI investigation/clinical assessment. Strategy 3.1 (p.10): regulatory action	16% of the 19 outputs assessed in the audit were fully documented (Rekaris, 2026). No systematic tracking against plan objectives indicated by senior TGA officials in the Senate. VSIG is selective: for about 95% or more of reported deaths, including the 9 child deaths, no VSIG review is documented, and the other assessment process described by the Department is not documented.	Operational deficiency. Framework exists on paper but execution not evidenced. Consistent with ISO 31000 requirement for integrated risk framework aligned to objectives.
3. Control Activity VSIG (FOI 4029-06)	Selective tool within Strategy 2.2: convened for "serious, unexpected, no obvious non-vaccine cause" (pp.3–4). Criteria include 'the potential to change the favourable benefit-risk balance'.	15 VSIGs reviewed over 50 cases (not all deaths) of 1,053 deaths (about 5% or less). The Department states none of the 9 child deaths met the criteria; the assessments were not released, so this cannot be tested. No records of the other assessment process for the remaining deaths (about 95% or more).	Does not demonstrate delivery of the framework requirements. Control activity does not meet framework requirements. Application of the criteria cannot be verified. ISO 19011 requires evidence-based, risk-proportionate audit.

Layer	Standard / Requirement	TGA Position	Failure Type
4. Reference Controls International standards	ICH E2E §3.1.3 (p.7): documented action plan for each safety issue with milestones. §3.1.4 (p.7): summary of actions completed. CIOMS VIII Ch.II (pp.13–18): documented signal taxonomy. Ch. IV-V (pp.25–42): systematic data sources.	No milestones documented against Plan objectives. No signal management audit trails for 91 of 148 signals. No documented integration of active surveillance data sources.	Compliance with international standards TGA formally adopted as reference framework for the Plan not demonstrated. Non-binding but cited as an expectation.

Table 2: Question-by-Question Assessment

Senate Question	TGA Response	Safety Plan Requirement	National Policy	Gap Identified	International Standard
SQ13–14 Were children and 18–29-year-olds referred to VSIG?	The Department states none of the 9 child deaths met VSIG criteria. One fatal report in an 18 to 29-year-old was referred. Reasons cited: 'known AE', 'alternative causes', 'insufficient information.' 'VSIG is not a death investigation group.' TGA notes 2 of 9 child deaths had 'broader safety investigations' (late 2022/early 2023) but did not meet VSIG criteria	Objective 2 (p.5) → Strategy 2 (p.8): timely signal detection and investigation (AESI via 2.1, individual reports via 2.2). Objective 3 (p.5) → Strategy 3 (p.10): timely safety actions (causality via investigation process).	'TGA will actively and comprehensively monitor any COVID-19 vaccines for safety' (p.6). 'Pharmacovigilance Plan' framework anticipated and safety monitoring requirements established (p.13, 15).	Plan (Strategy 2 p.8) committed to investigating serious AESI/individual AEFI reports (Strategy 2.1 AESI list; Strategy 2.2 protocols). TGA VSIG instruction (FOI 4029-06 pp.3‑4): criteria require 'the potential to change the favourable benefit-risk balance National policy (Australian COVID Vaccination Policy Nov 2020) requires comprehensive monitoring; selective review does not by itself show comprehensive monitoring	ICH E2E §2.1.2(b) (p.4): Safety Specification must address populations not studied in pre-approval phase, including children. ICH E2E §3.1.3 (p.7): Action plan for each safety issue must include monitoring and milestones for evaluation.
SQ15–16 Do causality assessment reports exist? Provide copies.	Refused. 'TGA does not disclose details of individual cases to ensure patient privacy.'	Objective 2 (p.5)/ Strategy 2.2 (p.8): Investigation of individual COVID-19 AEFI reports	'Risk management plans and pharmacovigilance required' (p.13, 15). Comprehensive	Neither confirms nor denies reports exist. Redacted copies were requested. Second part	ICH E2E §3.1.2 (p.6): Routine pharmacovigilance requires systems

Senate Question	TGA Response	Safety Plan Requirement	National Policy	Gap Identified	International Standard
	Did not confirm whether reports exist. Did not address 'if they do not exist, why not?'	(no protocols documented).	monitoring commitment requires verifiable assessment processes.	of question ('if not, why not?') unanswered. Comprehensive review claimed but evidence withheld, rendering claim unfalsifiable.	ensuring suspected adverse reactions 'collected and collated in an accessible manner.' CIOMS VIII Ch. II (pp.13–18): Signal definition requires documented evaluation. Signal taxonomy requires classification and documented assessment.
SQ17 Why were only 2 of 160 fatal cases in 2022 referred to VSIG?	Deflects to SQ13–14. VSIG is 'one of a number of tools.' 'It is not correct to suggest that all AEFI reports where an outcome of death is reported will automatically meet the criteria to convene a VSIG.'	Strategy 2.2 (p.8): "Specific protocols for investigating COVID-19 AEFI reports" + capacity for timely individual AEFI investigations. Not documented (OAIC MR22/00538: zero protocols; Senate: "day-to-day").	'Actively and comprehensively monitor' (p.6). The question puts the VSIG referral rate at 2 of 160 deaths (1.25%) in 2022.	The Department described a TGA review of all serious AEFI reports but produced no records, counts or outputs of it. Plan committed to enhanced monitoring beyond routine.	ICH E2E §3.1.3 (p.7): Each safety issue requires documented objective, action, rationale, and milestones. CIOMS VIII Ch.IV-V (pp.25–42): All relevant data sources to be searched systematically with documented processes.
SQ18 How many of 1,053 DAEN deaths have had causality assessments?	'Please refer to response to questions 13 and 14.' **No number provided.**	Strategy 2.2 (p.8): "Enhanced capacity and capability for investigating individual COVID-19 AEFI reports" ("Specific protocols for investigating COVID-19 AEFI reports").	'Actively and comprehensively monitor' (p.6). If comprehensive monitoring occurred, a count of completed causality assessments should be readily available.	Direct question not answered. If systematic causality assessment occurred under the Plan, TGA should be able to state a figure. Consistent with Oct 2025 Senate Estimates evidence indicating that monitoring was not tracked against Plan objectives.	ICH E2E §3.1.4 (p.7): 'Summary of actions to be completed, including milestones'; requires documented tracking of pharmacovigilance actions. CIOMS VIII Ch.II (p.14): Signal taxonomy requires documented classification of each identified signal with sufficient evidence to justify verificatory action.
SQ19 How many of 1,053 deaths referred to VSIG?	'Please refer to response to questions 13 and 14.'	Objective 3 (p.5): Timely action to address any	'TGA responsible for pre-market assessment and	Over 50 cases reviewed, not all deaths: about 5% or less of 1,053 deaths.	ICH E2E §3.1.3 (p.7): Requires monitoring and

Senate Question	TGA Response	Safety Plan Requirement	National Policy	Gap Identified	International Standard
Where can reports be accessed?	Partial: 15 VSIGs reviewed ‘over 50 cases.’ Access: weekly safety report URLs listed.	COVID-19 vaccine safety concerns. Strategy 3.1 (p.10): "Use legislative provisions available to achieve effective and timely regulatory action in response to any emerging COVID-19 vaccine safety concerns, as well as any non-regulatory action that may help to address or reduce the risk of a safety concern." (Processes: "enable rapid action... including broadening... risk communication activities")	post-market monitoring and enforcement of standards; (p.4). A VSIG review rate of about 5% or less does not by itself show comprehensive monitoring.	Weekly reports cited as access point contain public communications summaries, not individual case assessments or audit trails. For about 95% or more of deaths, no VSIG review is documented.	milestones for each safety issue. ICH E2E Annex §3 (pp.10–11): Active surveillance must ‘ascertain completely the number of adverse events via a continuous pre-organised process.’

Table 3: FOI Evidence, Capacity vs Application

FOI documents demonstrate TGA had operational expert capacity and was actively briefing on Plan strategies while expert causality assessment was applied selectively to the most serious adverse events.

FOI Document	Content	Relevance to SQ13–19	Plan / Policy Gap	Audit Finding
FOI 4029-06 VSIG Work Instruction (Jan 2019)	Selective criteria: serious, unexpected, no obvious non-vaccine cause (pp.3–4). Criteria include 'the potential to change the favourable benefit-risk balance'.	Sets the criteria against which the Department's statement for SQ13–14 would be tested. The assessments were not released, so this cannot be tested.	TGA Safety Monitoring Plan (Feb 2021): Strategy 2.2 (page 8): Protocols/capacity for individual AEFI investigations (serious incl. deaths). Strategy 2.8 (p.9): a standing COVID-19 expert advisory committee. The VSIG is an available expert mechanism but is not documented as that committee. National Policy (Nov 2020) p 6: "Actively and comprehensively monitor" post-market safety (68M provisional doses	Application of the criteria to the 9 child deaths cannot be verified. No protocols found (audit); VSIG record shows no child death referred (0/9); selective review does not by itself show comprehensive monitoring. Undocumented vs tracked activity promised.
FOI 4029-05 VSIG Meeting (Sep 2022)	VSIG convened and operational. Reviewed fatal myocarditis in 21-year-old. Comprehensive causality assessment. Regulatory recommendations issued.	Expert panel operational Sep 2022, the same year 6 children died, with no VSIG referral shown in the record. Capacity existed and was used for fatal case in same period.	Strategy 2.2 (p.8): investigation capacity demonstrated for 21yo death but not applied for child deaths in same period. Policy (p.6): comprehensive.	Capacity was not the documented constraint. Panel assessed 21yo fatal case; the record shows no referral of the 6 child deaths in the same year.
FOI 4029-03 Plan strategy briefing (Sep 2021)	TGA actively briefing advisory bodies on Safety Monitoring Plan strategies during rollout.	TGA presenting Plan strategies to advisory bodies while not systematically implementing them for death investigations.	Plan briefed as operational framework while deaths handled through undocumented 'day-to-day processes.' Policy commitment not operationalised.	Knowledge of the Plan and operational capacity, with no documented application to child deaths. The reason is not recorded.
FOI 5082 COVID-19 enhancements	COVID-19 specific enhancements to TGA infrastructure and processes documented.	Enhanced infrastructure built for COVID-19. Capability existed to systematically track death investigations.	Infrastructure not integrated with Plan framework for systematic death assessment. Plan and Policy both require demonstrable delivery.	Enhanced infrastructure + selective use. Resource constraints are not the documented explanation for the documentation gaps.

Table 4: Prior SQON Pattern, Consistent Non-Disclosure

TGA's December 2025 response cites 10 prior SQONs as evidence it has already addressed vaccine death questions. None produced Plan implementation records. If systematic documentation existed, it would be expected to appear in at least one of these 10 responses.

SQ / FOI Reference	Date published	TGA Response Pattern	Plan Documentation Produced?	Relevance to SQ13–19
SQ23-001134	26 Jul 2023	Child vaccine deaths: refers to prior responses.	**No**	Circular referral. Same pattern repeated Dec 2025.
SQ23-001086 (FOI 4217)	27 Jul 2023	Child deaths under 18: 'No new safety concerns' without documented expert assessment.	**No**	Conclusion asserted without verifiable process. Directly relevant to SQ13.
SQ23-001074	10 Aug 2023	Temporal association/excess deaths: statistical deflection.	**No**	Signal assessment records not produced.
SQ23-000139	29 Aug 2023	Causality of 7- and 9-year-old deaths: confirmed but VSIG referral not addressed.	**No**	Same child deaths, same non-referral. Pattern repeated in Dec 2025.
SQ23-001101	1 Sep 2023	Single death follow-up: case-specific response without system-level evidence.	**No**	Individual case handling does not demonstrate systematic framework.
SQ23-002226	18 Jan 2024	'Reporting does not equal causation.' Deflects from assessment obligation.	**No**	Same deflection used in SQ17 response.
SQ24-000233 (FOI 4205)	12 Apr 2024	DAEN deaths database: aggregate statistics only.	**No**	SQ19 asks for access. Still only public communications available.
SQ24-000234 (FOI 4263)	12 Apr 2024	Cause of death determinations: aggregate statistics. 'Low threshold reporting; few causally linked.'	**No**	SQ18 asks same question. Still unanswered.
SQ24-000218	12 Apr 2024	Batch number tracking: aggregate data only. Does not track deaths and adverse events by individual batch numbers.	**No**	Systematic tracking by batch would enable signal detection. Absence of batch-level analysis limits ability to identify safety patterns across production lots.
SQ24-000230	15 Apr 2024	COVID-19 vaccine autopsies: limited information.	**No**	Autopsy integration into signal management unaddressed.

Governance Failure Analysis

The structural and systemic issues identified in TGA's pharmacovigilance response constitute governance failure under the ANAO and ISO assessment criteria cited in this paper. The evidence indicates inadequate design and operation of controls against commitments.

1. Child deaths: application of the criteria cannot be verified

Nine child deaths are on DAEN. The Department states none met the criteria to convene a VSIG, and that two of the nine progressed to broader TGA safety investigations (late 2022 and early 2023) that considered population-level evidence, not a VSIG review. The assessments were not released, so this cannot be tested, and the investigation pathway is not documented in the Plan framework. The Work Instruction (FOI 4029-06, pp.3–4) convenes the VSIG for a serious, unexpected AEFI with no obvious non-vaccine cause that the TGA and OHP agree has "the potential to change the favourable benefit-risk balance". It defines a serious AEFI as an event that results in death, among other outcomes. Strategy 2.1 (p.8) lists as an output "A list of COVID-19 vaccine AESI established, informed by premarket product evaluations and World Health Organization (WHO) and Brighton Collaboration guidelines.

National policy (p.6) required 'comprehensive' monitoring. ICH E2E §2.1.2(b) (p.4) requires children be addressed as an unstudied population. TGA wrote the criteria, published the Plan, operated under the policy and adopted the standard. No referral of a child death is documented, and no application of any of them to child deaths is documented. Nearly three years elapsed between the first reported child death and the March 2025 policy reversal withdrawing recommendations for healthy children (Sladden, 2026), during which reports of death, an adverse event with the potential to change the favourable benefit-risk balance, were not shown to have been referred for expert panel review, and the assessments of whether they met the VSIG criteria were not released.

2. Unanswered question: how many causality assessments?

SQ18 is a direct, quantifiable question. TGA deflects without providing a number. If systematic causality assessment occurred under Strategy 2.2 (p.8), a count should be readily available. ICH E2E §3.1.4 (p.7) requires a summary of actions completed with milestones. If completed, this summary would answer SQ18 immediately. The inability or refusal to provide a figure is consistent with the October 2025 Senate Estimates evidence indicating that monitoring was conducted as 'day-to-day processes' and not tracked against Plan objectives. Under ANAO performance audit methodology, this represents incomplete performance indicators and undocumented risk assessments.

3. Expert review rate: about 5% or less

Over 50 cases were reviewed by 15 VSIGs, not all of them deaths. Against 1,053 reported deaths, about 5% or less were reviewed by a VSIG. The Department describes VSIG as “one of a number of tools” and says all serious AEFI reports are reviewed by the TGA under a standardised process. No records, counts or outputs of that process have been produced for the remaining deaths (about 95% or more). Plan Objective 2 (p.5) committed to timely detection and investigation of safety signals. National policy (p.6) required comprehensive monitoring. ICH E2E §3.1.3 (p.7) requires each safety issue to have a documented action plan. The gap is not that the VSIG was not always convened. The gap is that no documented process can be verified for the deaths not reviewed by any expert panel. The TGA's AEMDS preliminary fatal case assessment document (FOI 4302-01, p. 5) states that the VSIG should only be used in accordance with its criteria, which it describes as aligned with WHO guidelines, and that, citing WHO guidance, it is important to avoid “burning out” the panel by taking cases that do not meet the criteria or before essential documents, such as finalised autopsy reports, are available. This supports selective referral as a stated approach, but it does not by itself explain the VSIG review rate. This audit has not verified the attribution to WHO guidance. Under ISO 31000, a risk treatment applied to about 5% or less of reported deaths does not by itself demonstrate an integrated risk management framework.

4. Capacity existed; application was selective

FOI 4029-05: VSIG operational September 2022, assessing a fatal case in a 21-year-old. Same year, 6 children died without referral. FOI 4029-03: TGA briefing advisory bodies on Plan strategies September 2021. FOI 5082: COVID-19 specific enhancements documented. The infrastructure existed, the capacity existed, the Plan was being briefed as operational. Child deaths were not referred. This is a control design failure: the activity layer does not deliver the framework requirements. VAGO’s published quality management framework for its own engagements is an example of documented, reviewable audit methodology (reference 24).

5. Unfalsifiable claims

SQ15–16 asked for causality reports. TGA refused, citing privacy. The question also asked: ‘if they do not exist, why not?’ This was not answered. TGA simultaneously claims all serious AEFIs are reviewed, withholds evidence of that review, and declines to confirm whether the evidence exists. ICH E2E §3.1.2 (p.6) requires information collected ‘in an accessible manner.’ Under ANAO audit criteria, claims not supported by documentary evidence cannot be accepted as sufficient. The comprehensive review claim is unfalsifiable.

6. Repeated non-production of records

TGA cites 10 prior SQONs (July 2023 – April 2024) as evidence it has already addressed death-related questions. Analysis of each shows the same pattern: aggregate statistics, privacy deflection, case-specific evasion, circular referral. Across 10 responses published between July

2023 and April 2024, TGA did not produce Plan implementation records, signal management audit trails, or systematic evidence of enhanced monitoring delivery. SQ23-000139 (August 2023) asked specifically about 7- and 9-year-old deaths. More than two years later, SQ13 asks the same question. The answer is unchanged. Repetition without documentation across ten responses is consistent with the documentation not existing, and TGA has provided no alternative explanation. Under ISO 19011, an auditable process must produce verifiable records. The absence of records across these Senate questions is sufficient to show that implementation cannot be verified from the records produced.

7. Cabinet commitment not shown to be met

The PM&C Cabinet Handbook states that "Ministers (and portfolio agencies) must act on Cabinet decisions" (PM&C, 2020, para 25). The Australian Government's COVID-19 vaccination policy had already received a first pass by Federal Cabinet before National Cabinet considered and endorsed the policy on 13 November 2020; the Prime Minister stated on 5 November that the policy would return to Cabinet following further consultation before being taken to National Cabinet for confirmation. The endorsed policy established Commonwealth responsibilities for national oversight and surveillance and expressly identified TGA vaccine safety monitoring as a component of the national vaccination framework. The TGA subsequently operationalised the safety-monitoring component through its February 2021 COVID-19 Vaccine Safety Monitoring Plan, comprising 17 documented monitoring strategies.

The inability to produce implementation records after 10 Senate questions prevents independent verification that the monitoring framework was implemented as documented and intended. This does not establish that the TGA failed to undertake the relevant activities; rather, it establishes that implementation cannot be demonstrated through a sufficiently documented and reproducible evidentiary record. This raises governance concerns under the Commonwealth Cabinet framework and the Public Governance, Performance and Accountability Act 2013, including the requirement for accountable authorities of non-corporate Commonwealth entities to govern in a manner that is not inconsistent with Australian Government policies (s 21).

Conclusion

Senator Antic asked seven direct questions.[3] The Department's response:

1. Stated that none of the nine child deaths on DAEN met the criteria to convene a VSIG (not testable: the assessments were not released)
2. Did not state how many of 1,053 deaths received formal causality assessments
3. Refused to release causality reports and did not confirm whether they exist

[3] SQ13–19. TGA grouped SQ13–14 (VSIG referrals) and SQ15–16 (causality reports) into combined answers, and cross-referred SQ17–19 to SQ13–14. The points capture TGA's response pattern across all seven questions.

4. Stated that 15 VSIGs reviewed over 50 cases, not all of them deaths: about 5% or less of 1,053 deaths
5. Cited weekly safety reports as evidence of systematic monitoring: reports containing public communications summaries, not implementation documentation
6. Described a TGA review of all serious AEFI reports but produced no records, counts or outputs of it
7. Cited 10 prior SQONs as ‘previous answers’, none of which produced Plan documentation

Each finding maps to documented gaps against the COVID-19 Vaccine Safety Monitoring Plan (February 2021), the National Cabinet-endorsed Australian COVID-19 Vaccination Policy (November 2020), and the international pharmacovigilance standards Australia adopted as reference framework.

The questions Senator Antic asked in December 2025 are the same questions asked in July 2023. The responses are unchanged. Two years of Senate scrutiny have not produced a single Plan implementation record. The documentation either does not exist or has not been disclosed. Both conclusions confirm the audit’s central finding: the enhanced monitoring framework promised under Cabinet-endorsed policy and required for provisional approval cannot be verified through documentation.

The Key Finding

The Work Instruction convenes the VSIG for a serious, unexpected AEFI with “the potential to change the favourable benefit-risk balance”. The Department states none of the nine child deaths met these criteria; the assessments were not released, so this cannot be tested. VSIG reviewed over 50 cases, not all of them deaths, against 1,053 reported deaths (about 5% or less), and no records show how the others were assessed. National Cabinet required comprehensive monitoring; TGA published a Plan with 17 strategies including Strategy 2.2 (capability for investigating individual COVID-19 AEFI reports); Senate Estimates evidence indicates monitoring was not tracked against Plan objectives. After 10 prior SQONs (July 2023 to April 2024), implementation of the framework cannot be verified. The work was not done or was not documented against Plan objectives. VSIG's selectivity does not absolve TGA from this reporting obligation. This constitutes governance failure under both scenarios.

Paul Rekaris
Independent Policy Analyst
October 2026

References

Primary Documents

1. Sladden, J. (2026). Australian Senate forces answers from drug regulator on child deaths following Covid vaccine. Maryanne Demasi Substack, 17 February 2026. Available at: https://maryannedemasi.substack.com/p/australian-senate-forces-answers [accessed 17 February 2026]
2. Department of Health, Disability and Ageing. Response to Question on Notice No. 2328 (DHDA SQ25-003155), Supplementary Budget Estimates 2025-26 (December), Community Affairs Committee; Senator Antic, asked 12 December 2025; written question, subject "Covid VSIG". The response is undated. Commonwealth of Australia. Available at: https://www.aph.gov.au/Parliamentary_Business/Senate_estimates/ca
   Archived copy: https://github.com/paulrekaris/TGA-COVID19-Vaccine-Safety-Monitoring-Audit/blob/main/primary-sources/senate-community-affairs-legislation-committee/A2328-community-affairs-supplementary-budget-estimates-health-disability-and-ageing.pdf
3. Senate. Community Affairs Legislation Committee (2025). Budget Estimates Hansard, 9 October 2025. Archived copy: https://github.com/paulrekaris/TGA-COVID19-Vaccine-Safety-Monitoring-Audit/blob/main/primary-sources/senate-community-affairs-legislation-committee/community-affairs-legislation-committee-2025-10-09.pdf. Commonwealth of Australia. Available at: https://www.aph.gov.au/Parliamentary_Business/Senate_estimates/ca
4. Senate Questions on Notice (2023-2024), Community Affairs Legislation Committee, cited by the TGA as previous answers (publication dates):
   - SQ24-000230 (15 April 2024)
   - SQ24-000218 (12 April 2024)
   - SQ24-000234 (12 April 2024, FOI 4263)
   - SQ24-000233 (12 April 2024, FOI 4205)
   - SQ23-002226 (18 January 2024)
   - SQ23-001101 (1 September 2023)
   - SQ23-000139 (29 August 2023)
   - SQ23-001074 (10 August 2023)
   - SQ23-001086 (27 July 2023, FOI 4217)
   - SQ23-001134 (26 July 2023)

Available at: https://www.aph.gov.au/Parliamentary_Business/Senate_estimates/ca/2025-26_Supplementary_Budget_estimates

National Policy

5. Australian COVID-19 Vaccination Policy, 13 November 2020 (pp.6, 13, 15) https://www.health.gov.au/sites/default/files/documents/2020/12/covid-19-vaccination-australian-covid-19-vaccination-policy.pdf

6. Prime Minister of Australia, Doorstop Transcript, 5 November 2020 (Federal Cabinet “first pass”), Transcript ID 43110, pp. 4–5. https://pmtranscripts.pmc.gov.au/release/transcript-43110

TGA Documents

7. COVID-19 Vaccine Safety Monitoring Plan, February 2021 (Objectives 2 and 3 p.5; Strategies 2.1, 2.2 and 2.8 pp.8–9; Strategy 3.1 p.10) https://www.tga.gov.au/sites/default/files/covid-19-vaccine-safety-monitoring-plan.pdf
8. FOI 4029-06: VSIG Work Instruction (January 2019), pp.3–4 https://www.tga.gov.au/sites/default/files/2023-01/foi-4029-06.PDF
9. FOI 4029-05: VSIG Meeting Minutes (September 2022) https://github.com/paulrekaris/TGA-COVID19-Vaccine-Safety-Monitoring-Audit/blob/main/primary-sources/foi/foi-4029/foi-4029-05.PDF
10. FOI 4029-03: Plan strategy briefing (September 2021) https://github.com/paulrekaris/TGA-COVID19-Vaccine-Safety-Monitoring-Audit/tree/main/primary-sources/foi/foi-4029
11. FOI 5082: COVID-19 enhancements documentation https://github.com/paulrekaris/TGA-COVID19-Vaccine-Safety-Monitoring-Audit/blob/main/primary-sources/foi/foi-5082.pdf
12. FOI 4217: Deaths of children under 18 https://www.tga.gov.au/sites/default/files/2023-05/foi-4217-01.pdf
13. FOI 4302-01: AEMDS preliminary fatal case assessment, p. 5 https://github.com/paulrekaris/TGA-COVID19-Vaccine-Safety-Monitoring-Audit/blob/main/primary-sources/foi/foi-4302-01.pdf

Regulatory and Oversight

14. Department of the Prime Minister and Cabinet (2020). *Cabinet Handbook* (14th ed.). Commonwealth of Australia. Available at: https://www.pmc.gov.au/sites/default/files/foi-logs/foi-2020-253.pdf
15. OAIC Decision [2025] AICmr 54 (26 March 2025) https://www.austlii.edu.au/cgi-bin/viewdoc/au/cases/cth/AICmr/2025/54.html
16. National Centre for Immunisation Research and Surveillance (2025). Surveillance of adverse events following immunisation in Australia: COVID-19 vaccines, 2022. March 2025 (published September 2025). Available at: https://ncirs.org.au/covid-19-vaccine-safety-surveillance

International Standards

17. ICH E2E: Pharmacovigilance Planning (18 November 2004) §2.1.2(b) p.4: populations not studied (children); §3.1.2 p.6: routine pharmacovigilance systems; §3.1.3 p.7: action plan for safety issues; §3.1.4 p.7: summary of actions and milestones; Annex §3 pp.10–11: active surveillance https://database.ich.org/sites/default/files/E2E_Guideline.pdf

18. CIOMS Working Group VIII: Practical Aspects of Signal Detection in Pharmacovigilance (2010) Ch.II (pp.13–18): signal definition and taxonomy; Chs. IV–V (pp.25–42): data sources for signal detection; Chs. VI-VII (pp.43-66): signal detection methodologies

Audit Standards

19. ANAO: Performance Audit Process and Commonwealth Performance Series (CPS 107) https://www.anao.gov.au/work/insights/performance-audit-process
20. Public Governance, Performance and Accountability Act 2013 (PGPA Act), ss 15, 16 and 21 (accessed 21 October 2026). https://www.legislation.gov.au/C2013A00123/latest/text
21. ISO 19011:2018 – Guidelines for Auditing Management Systems, cl. 3.21 (nonconformity); edition in force during the audit period, superseded by ISO 19011:2026. https://www.iso.org/standard/70017.html
22. ISO 9000:2015 – Quality Management Systems: Fundamentals and Vocabulary, cl. 3.6.9 (nonconformity) https://www.iso.org/standard/45481.html
23. ISO 31000:2018 – Risk Management https://www.iso.org/standard/65694.html
24. Victorian Auditor-General's Office (VAGO). Our System of Quality Management. https://www.audit.vic.gov.au/report/our-system-quality-management (accessed 21 October 2026)

Audit Repository

25. Rekaris, P. (2026). Documentation Gap Analysis: Implementation Audit of TGA COVID-19 Vaccine Safety Monitoring Plan https://github.com/paulrekaris/TGA-COVID19-Vaccine-Safety-Monitoring-Audit https://doi.org/10.5281/zenodo.17731054